

	EMERGENCY MEDICAL SERVICES AGENCY	POLICY NUMBER	736
	A Division of the Merced County Department of Public Health	Effective Date:	11/2025
Authority: Health and Safety Code, Division 2.5, and California Code of Regulations, Title 22, Division 9, Chapter 4, 1797.220		Initial Date:	09/2018
		Review Date:	04/2028

MEDICATION POISONINGS	
Beta Blockers - Metoprolol/Lopressor/Toprol, Labetalol/Esmolol, Carvedilol/Coreg, Atenolol/Tenormin (amongst others). Calcium Channel Blockers - Norvasc/Amlodipine, Diltiazem/Cardizem, Nifedipine/Procardia, Verapamil/Calan (amongst others). Cyclic Antidepressants - Amitriptyline, Doxepin, Imipramine, Nortriptyline, Trimipramine (amongst others).	
ADULT	PEDIATRIC
BLS	
Assess Vitals / Obtain SpO ₂ / Oxygen - Titrate to SpO ₂ of 94% or higher / Secure Airway - Assist Ventilations if needed. Blood Sugar Check – If indicated.	
ALS	
Follow BLS procedures if applicable. Obtain 4-Lead ECG. Vascular Access – Establish large bore IVs - IV/IO rate as appropriate, Refer to EMS Policy #738 – Non-Traumatic Shock.	
Beta Blockers	
For patients with SBP < 90 mmHg (Adult) or hypotension per length-based assessment tape or application (Pediatric) and bradycardia:	
Push Dose Epinephrine - 10mcg slow IV/IO (10mcg/ml) Repeat every 1-5 minutes until SBP is > 90mmHg	Refer to EMS Policy #706 – Bradycardia.
<u>If patient is not responding to previous treatment:</u> Glucagon - 2 mg IVP	
<u>If patient is not responding to previous treatment:</u> Transcutaneous Pacing	Base Hospital Physician Orders Only Make Base Contact for further treatment, consultation, and orders.

Signatures on File

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EMS Administrator

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EMS Medical Director

Calcium Channel Blockers

For patients with SBP < 90 mmHg (Adult) or hypotension per length-based assessment tape or application (Pediatric) and bradycardia:

Calcium Chloride – 1000 mg slow IVP (1 ml/minute) over 10 minutes OR until SBP > 90 mmHg.

Refer to EMS Policy #706 – Bradycardia.

If patient is not responding to previous treatment:

Push Dose Epinephrine - 10mcg slow IV/IO (10mcg/ml)

- Repeat every 1-5 minutes until SBP is > 90mmHg

Base Hospital Physician Orders Only

Make Base Contact for further treatment, consultation, and orders.

If patient is not responding to previous treatment:

Transcutaneous Pacing

Tricyclic Antidepressant

For Cardiac Dysrhythmia or QRS complex > 0.10 seconds:

Sodium Bicarbonate - 1 mEq/kg slow IVP

- Repeat as necessary

If patient is not responding to previous treatment and is hypotensive with SBP < 90 mmHg (Adult) or per length-based assessment tape or application (Pediatric):

Push Dose Epinephrine - 10mcg slow IV/IO (10mcg/ml)

- Repeat every 1-5 minutes until hypotension is resolved

Seizure Control

Midazolam – 2 mg IV/IO OR 5 mg IN/IM

- Titrate IV/IO in **1 mg** increments for seizure control, max dose 5 mg
- May repeat IN/IM once after 10 minutes if seizures continue

Midazolam – 0.1 mg/kg IV/IO OR 0.2 mg/kg IN/IM

- Max dose 5 mg
- May repeat once after 10 minutes if seizures continue

Special Considerations

Administer fluid boluses with caution due to the high incidence of pulmonary edema in tricyclic overdose patients.

Most tricyclic overdose seizures are short in duration and may not require the administration midazolam.